Green Paper 03 — Viability and the Conditions for Care

What care needs in order to continue without consuming its source

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Abstract

Care is often described as a virtue, an intention or a personal quality. Yet care does not become durable through goodwill alone. It requires time, material sufficiency, attention, trust, boundaries, recovery and access to repair. When those conditions are absent, people may continue to care for a while, but the work is increasingly carried through exhaustion, self-erasure and hidden transfers of cost.

This paper approaches viability as a quiet ethical and institutional threshold: the capacity to continue a life-serving commitment without progressively weakening its source. It asks what changes when depletion is treated not as proof of devotion, but as information about the design of a field. It proposes that ecological integrity, steward viability and the quality of governance must each remain inhabitable; strength in one cannot compensate for collapse in another. Care becomes viable when it can circulate, when limits are legible, when repair is possible and when responsibility does not depend on heroism.

There is no shame in depletion.
But there is clarity in tending.
The soil wants to hold us again.
Will we let her?

1. Care begins in conditions

Care is real as feeling, gesture and intention. It is also material. Someone must have enough time to notice. A body must have enough capacity to respond. A relationship must be able to hold truth without making honesty too dangerous. An institution must leave room for judgment, repair and pause. A landscape must be allowed to regenerate after what has been taken from it.

This means that care is never only a question of whether people care. It is also a question of whether the conditions around them allow care to become practice.

A person can be deeply committed and still be unable to carry another demand. A team can share humane values and still reproduce neglect when its pace makes attention impossible. An institution can speak the language of care while depending on unpaid availability, emotional overextension or the quiet absorption of structural failure by a few conscientious people. Benevolent intention does not cancel these realities. Sometimes it helps to conceal them.

The practical question is therefore not simply: *Do we care?* It is: *What does care require here, and are those requirements actually present?*

This shift matters because it moves the burden away from character alone. It does not remove personal responsibility. It places responsibility inside a larger field of capacity, access, rhythm and design. It allows us to see that an apparent failure of compassion may be a failure of time; that a breakdown in cooperation may be a failure of trust; that numbness may be the body’s response to prolonged demand; and that an organisation’s dependence on exceptional individuals may be evidence of institutional fragility rather than excellence.

Care begins in the wish to protect life. It becomes viable only when life is not continually sacrificed in order to perform it.

2. Viability is enoughness without depletion

Viability is not perfection, comfort or the absence of strain. Living systems meet difficulty. People become tired. Seasons of intensity occur. Resources are sometimes scarce, and meaningful commitments can ask much of us.

The distinction is not between effort and ease. It is between effort that can be metabolised and demand that steadily consumes the capacity to respond.

In this paper, viability means sufficient capacity to continue a life-serving flow without progressively weakening its source. The source may be a body, a relationship, a community, an institution, a watershed or a living landscape. The question is not whether the source is untouched. Every real exchange changes what participates in it. The question is whether the exchange leaves open a credible path of renewal.

This makes viability a threshold rather than a promise of endless continuity. A viable field can encounter disruption and require repair. It can reduce its scope. It can ask for help. It can pass responsibility from one steward to another. It can change form or end a particular activity in order to protect the life that activity was meant to serve.

Enoughness is central here. Enough time to listen before deciding. Enough material support that care is not financed by private collapse. Enough continuity to form trust. Enough redundancy that one person’s illness does not become a crisis for everyone. Enough freedom to say no before refusal arrives as breakdown. Enough honest information to recognise when the field has crossed from generative effort into extraction.

Viability does not ask whether everything can continue. It asks whether what continues can do so truthfully.

3. The hidden subsidy of care

Many systems appear to function because someone absorbs what the system does not account for.

A family member becomes permanently available. A nurse skips recovery. A teacher carries the distress that the timetable cannot hold. A volunteer turns intermittent commitment into invisible administration. A community organiser becomes the memory, mediator and emergency response of an entire field. A living landscape is treated as if regeneration were automatic and limitless.

These contributions may be freely given, meaningful and loving. The problem begins when a gift becomes an unspoken requirement, or when the continuation of a system depends on costs that remain unnamed because naming them would reveal that the system is not viable on its stated terms.

We can call this a hidden subsidy of care: the uncounted time, attention, health, emotional labour, ecological capacity and relational repair that keep an activity alive while disappearing from its account of itself.

The subsidy is not hidden merely because nobody measures it. It is often hidden by moral language. Exhaustion becomes commitment. Constant availability becomes generosity. The inability to withdraw becomes loyalty. A breached boundary becomes proof that the work matters. When this happens, care is recruited to legitimise its own depletion.

Burnout is not a badge of honor — it is a boundary breached.

This line is not a diagnosis of every exhausted person, nor does it make boundaries simple. It offers a different reading of evidence. Depletion is not shameful, and it is not automatically noble. It is information. It may tell us that a responsibility is too concentrated, that material support is missing, that the pace is incompatible with attention, that the mandate has expanded without consent, or that a person has remained beyond the point at which care could still move freely.

An honest account of care therefore includes what the work draws upon, who is carrying its unpriced remainder, and what forms of renewal are actually available. Without that account, apparent efficiency may be organised exhaustion.

4. Care must be able to circulate

Care becomes fragile when it has only one direction.

If one person always notices, remembers, steadies, translates and repairs, the field may feel held while becoming increasingly dependent on that person’s overextension. If those receiving care are never allowed agency, contribution or refusal, care can become paternalism. If the giver cannot receive support without shame or loss of authority, care becomes an identity that traps the person performing it.

Viable care circulates. This does not mean that every exchange is equal at every moment. A child and an adult do not carry the same responsibility. A person in acute distress may not be able to reciprocate. Historical and structural inequalities cannot be resolved by pretending that everyone enters a relation with the same resources. Circulation means something quieter: that the field does not permanently convert some lives into sources and others into destinations.

Care may circulate as rest, money, time, recognition, practical help, shared memory, permission to withdraw, protection from demand or a transfer of responsibility. It may be reciprocal without being immediate or symmetrical. What matters is that those who sustain the field are themselves included among the lives the field is designed to sustain.

This also protects care from capture. Support must not purchase obedience, gratitude, access or control. Receiving care does not erase autonomy. Giving care does not establish ownership. A life-serving relation makes room for the recipient to remain a subject, including the freedom to question the form of care, refuse it or seek it elsewhere.

There is no center of power — only centers of care.

Read institutionally, this is not a denial that power exists. It is a proposition about how power might be held: close to responsibility, distributed across several capable centres and answerable to the lives affected by its decisions. Care that cannot be questioned easily becomes command. Care that can receive feedback without retaliation has begun to become a commons.

5. Three realities that cannot compensate for one another

Projects devoted to ecological or social good are often evaluated through a single dominant measure: their visible impact, their financial survival, their reach, their scientific promise or the moral urgency of their purpose. Viability asks for a more demanding reading.

At least three realities must be considered together:

1. **Land and life.** What is happening to the ecological field, the beings involved and the material sources upon which the work depends?
2. **Steward viability.** Can the people carrying the work sustain their health, relationships and livelihoods over time?
3. **Governance and commons.** Are decisions, burdens, knowledge and benefits held in ways that preserve trust, consent, accountability and the possibility of repair?

These realities are related, but they are not interchangeable. Ecological benefit cannot justify the routine depletion of stewards. Healthy finances cannot compensate for damaged land. A loving group culture cannot make an unaccountable decision structure safe. Strong governance cannot turn an extractive ecological practice into care.

In practical terms, a red condition in one dimension cannot simply be averaged away by green conditions in the others. It becomes a signal to pause, reduce scope, repair or reconsider the form of the work. This is especially important where a compelling mission encourages people to override what their bodies, relationships or surroundings are reporting.

The threshold comes before growth. Before asking whether a field can expand, replicate or produce more, we ask whether its foundational conditions are alive. This is not a rejection of ambition. It is a protection against scaling the very pattern the work was created to change.

Steward viability therefore has the force of a veto. If the people who hold continuity are becoming chronically depleted, expansion slows or stops. The pause is not a punishment, and it need not become permanent. It is a form of truthfulness about the present capacity of the field.

6. Institutions that make care possible

Grace is not an afterthought.
It is the blueprint.
Systems do not have to be cold.
They can be kind.

Kind systems are not systems without standards, conflict or consequence. They are systems whose standards remain connected to the realities of living beings.

An institution makes care more viable when it protects time for attention rather than treating attention as friction. It creates routes for early concern, so people do not have to wait for crisis before they are believed. It distinguishes urgent need from manufactured urgency. It makes responsibility visible and transferable. It budgets for maintenance, coordination and repair rather than funding only the most visible output. It allows decisions to be revisited when their effects differ from their intentions.

It also refuses to make compassion an informal substitute for infrastructure. A caring colleague cannot permanently compensate for inaccessible procedures. A devoted steward cannot replace adequate staffing. A listening session cannot repair a field if the people affected have no influence on the decision that harmed them. Relational warmth matters, but it must not be used to soften conditions that remain structurally untenable.

Institutional care can be tested in ordinary moments. Can someone become ill without the whole field collapsing? Can a person raise a concern without being recast as disloyal? Can responsibility be handed over without the loss of all tacit knowledge? Is rest anticipated, or granted only after visible breakdown? Are those closest to the consequences able to shape the response? Is there somewhere for unresolved harm to go?

These questions turn care from an aspiration into an architecture. They ask not for constant gentleness, but for conditions in which human beings can remain responsive without being consumed by responsiveness.

7. Planetary care without heroism

The scale of ecological and social crisis can make ordinary limits feel morally inadequate. When species disappear, climates destabilise and people live under violent or degrading conditions, rest may appear self-indulgent. The urgency is real. So is the danger of building a response that reproduces the logic of expendability.

No individual nervous system can hold the whole planet. No small organisation can answer every need opened by its work. No generation can repair all accumulated harm at once. Planetary care becomes practicable when responsibility is situated: this place, these relations, this mandate, this period of time, these resources, this next truthful act.

Situated responsibility is not a retreat from the whole. It is how the whole becomes reachable without becoming an abstraction that consumes the people trying to serve it. Limits allow commitments to become dependable. A bounded promise may be smaller than an unlimited declaration, but it can be inhabited, evaluated and renewed.

This requires collective forms. Burdens can be rotated. Knowledge can be shared. Institutions can provide long memory without requiring one person to become the archive. Material support can follow responsibility. Networks can redistribute demand when one node is under strain. Public systems can protect the conditions of care rather than relying on private sacrifice to repair what policy has neglected.

Let us walk slow enough to notice what never shouts.

Planetary care includes what is easily missed: soil losing vitality, the gradual narrowing of a habitat, a volunteer who has stopped speaking, the administrative burden that excludes a small community, the relationship that remains polite after trust has gone. Not all urgency is loud. Some of the most consequential changes arrive as attrition.

To notice them, care needs pace. To respond, it needs shared capacity. To continue, it needs permission not to become heroic.

8. When care means stopping

Care is commonly associated with continuation: keeping a person safe, sustaining a relationship, preserving an institution, protecting a habitat or helping a project survive. Yet viability cannot mean that every existing form must be maintained.

Sometimes a programme has completed its purpose. Sometimes an organisation has become a vessel for protecting itself rather than serving its mandate. Sometimes a relationship can no longer remain safe in its present form. Sometimes continued intervention prevents an ecosystem from finding another equilibrium. Sometimes the most caring act is to release a role, close a process, reduce a promise or allow an ending to be grieved.

Stopping is not automatically care. Withdrawal can abandon people and externalise cost. Endings require responsibility: notice, honest communication, transition, preservation of relevant knowledge and attention to those who cannot simply move on. But the refusal to end can also become a form of harm when continuity is valued above life.

Viability therefore belongs not only to endurance but to discernment. What is the work actually protecting? Does its present form still serve that life? What would be released if the form changed? What would be placed at risk? Who carries the transition?

Care does not promise that nothing will be lost. It asks that loss is not hidden, casually transferred or confused with failure when a truthful ending is what allows life to continue elsewhere.

9. A provisional grammar of viable care

The following propositions are offered as a working grammar rather than a completed doctrine:

- Care is a capacity of a field, not only a quality of a person.
- Good intention cannot compensate for conditions that make attention, consent or repair impossible.
- Depletion is information. It should be met with curiosity and tending, not shame or automatic praise.
- A field is not viable when its visible success depends on invisible human or ecological loss.
- Those who carry care must be included among those the field is designed to sustain.
- Care may be reciprocal without being immediate, equal or symmetrical, but it must not permanently divide life into sources and recipients.
- Support does not purchase authority over the person who receives it.
- Ecological integrity, steward viability and accountable governance are non-compensatory conditions.
- When a foundational condition turns red, expansion pauses while the field listens and repairs.
- Rest, maintenance and transition are part of the work, not rewards granted after it.
- A smaller inhabitable promise may carry more care than a larger uninhabitable one.
- Sometimes protecting continuity requires a change of form. Sometimes protecting life requires an ending.

These propositions do not remove ambiguity. Care remains contextual and contestable. What restores one person may overwhelm another. What protects a local field may transfer harm elsewhere. A boundary can preserve capacity or conceal avoidance. A pause can enable repair or become a way of postponing accountability.

For that reason, viable care cannot be reduced to compliance with a list. It requires repeated contact with consequences. It asks whether the people and places involved can speak back to the form that claims to care for them. It keeps design open to correction.

10. Wagers and open questions

This paper rests on several wagers:

- that care becomes more dependable when its material and institutional conditions are made explicit;
- that protecting steward viability strengthens rather than weakens long-term responsibility;
- that naming hidden subsidies can reveal where apparent success is being financed by depletion;
- that non-compensatory thresholds can prevent urgent missions from overriding the life they intend to serve;
- that bounded, shared responsibility is more capable of planetary care than permanent heroism;
- and that a field able to pause, repair, redistribute or end is often more alive than one compelled to continue unchanged.

The wagers leave questions that cannot be settled in advance:

1. How can a community recognise depletion early without turning every fluctuation in capacity into a crisis?
2. Who gets to determine when a threshold has been crossed, especially where power and risk are unevenly distributed?
3. How can institutions resource rest and repair without making people prove collapse before support becomes available?
4. What forms of accounting can reveal hidden human and ecological subsidies without reducing care to what is measurable?
5. How can responsibility circulate while preserving continuity, skill and trust?
6. When does a boundary protect care, and when does it protect privilege from accountability?
7. What would it mean for public systems to treat care capacity as essential infrastructure?
8. How do we end a form responsibly when others still depend on it?

The questions are not an appendix to care. They are part of its practice. A viable field does not know everything in advance. It maintains enough honesty, time and relationship to remain answerable to what its actions reveal.

Textual provenance

The italicised passages are drawn from the *Gaia Bloom Constitution*, developed within the AbunDance artistic inquiry in 2025, particularly the scrolls *Healing the Soil*, *Grace in Systems*, *Spiral Governance* and *Caretaking the Invisible*. They function here as poetic source material: constitutional imagination entering a wider ethical and institutional inquiry. The prose does not treat the passages as evidence or translate the Gaia Bloom ontology wholesale. It lets their propositions meet the later Spiralweb corpus, practical experience and open questions.

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